



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلي حسن حسين حسن
المحافظة : الجيزة
رقم البطاقة : 25612302101604 رقم التليفون : 01090600515
العنوان : 11 ش علي بدران
العمر (تاريخ الميلاد) : 30-12-1956 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
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أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ليلي حسن حسين حسن National ID: 25612302101604	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

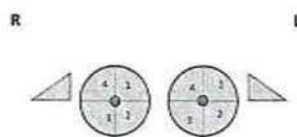


		جمهورية مصر العربية مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية		08/05/2026
MDT FORM				
Name: Lilaa Hsna Huseen Ali		MRN: 5497		
Age: 70 Y		ID: 25612302101604		
Menopausal Sta: postmenopausal		Phone: 01157196077		
NRS SCORE:		Marital Status:		
BMI:		Family History:		
Gender:		Reason Of MDT:		
CO Morbidity:		MBC 1st line		
Surgical History: MRM		ECOG Performance Status 0 1 2 3 4		

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** 1+ **KI67:**



T N M

History:

Diagnosed in 2022 with locally advanced Rt breast HR+ Her2- BC...received Neoadjuvant ET with sluggish response...Rt MRM...adjuvant ch...PORT...ET (AI).

Pt presented to us giving history of abdominal pain & vomiting...abdominal imaging revealed HFLs...PET/CT scan revealed osseous & hepatic deposits...biopsy from HFL revealed adenoca GATA3+, ER8/8 PR8/8 Her2 1+.

Sonomammography:**Biomarker/Molecular****Mets Workup:**

PET/CT 29/6/2026...Clear right MRM bed.

BilobarFDG avid HFLs.

Low grade FDG avid pleural based right pulmonary consolidative patch

Heterogenous low grade FDG avid omento peritoneal thickening with fat smudging, scattered omentanodules as well as deep umbilicus nodule.

ForMRI correlation.

Mixed FDG avid lytic and sclerotic osseous deposits.

Follow up PET/CTas clinically warranted would be advisable.

Recommendations:

for urgent MDT to shift to Faslodex/Ibrance.

MDT Outcome & Plan

shift to Faslodex/Ibrance + zometa q 3 mths.

Clinical Staging:**Pathological Staging:****SIGNATURE**

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين			

Name	Mrs. Laila Hassan Hussein Hassan		
Ref. By Prof. Dr.	Ashraf Mounir	Date	29/6/2026
ID	9926011151	Acc. No.	240322184

MULTI-SLICE PET/CT EXAMINATION OF THE BODY:

Clinical History:

- A69-year-old patient diagnosed as post-managed Rt. breast cancer.
- PET/CT requested for status evaluation.

Technique:

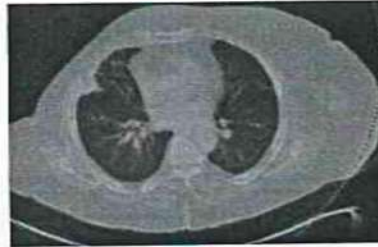
- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 300 MBq of 18 F-FDG.
- MSCT performed and used for attenuation correction, anatomical localization and diagnosis.
- Max. Variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.

Findings

Chest, both breasts and related LNs

(site of 1ry):

- Clear right MRM bed with no significant FDG avid lesion could be seen in the operative bed to suggest residual/recurrence.**
- Unremarkable Lt. breast.
- Low grade FDG avid pleural based right pulmonary consolidative patch SUV max~3.9
- No pathologically enlarged or dense tracer fixing related LNs seen.
- No evidence of pleural or pericardial effusion noted.

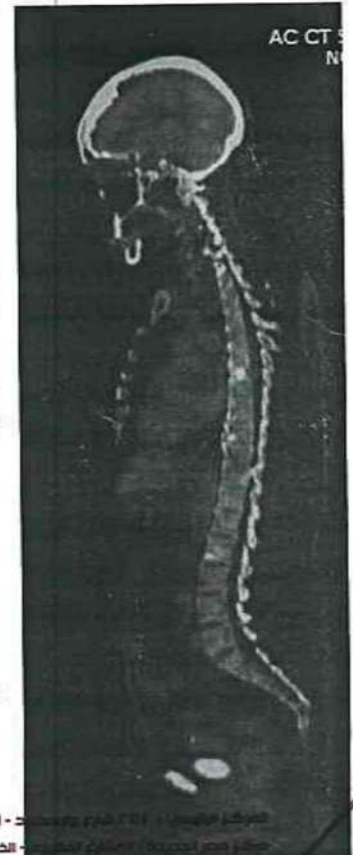


Head and Neck:

- Bilateral symmetrical FDG distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
(N.B: it may kindly be noted that some brain lesions not to be apparent on PET/CT and MRI is better for local assessment, if clinically warranted).
- Normal appearance of the nasopharynx, oropharynx and larynx.
- Normal appearance of thyroid and salivary glands.
- No pathologically enlarged or dense tracer fixing cervical LNs seen.

The Musculo-skeletal System:

- Mixed lytic and sclerotic over spine as well as right infra acetabular portion SUV max~8**
- Spine degenerative changes noted.

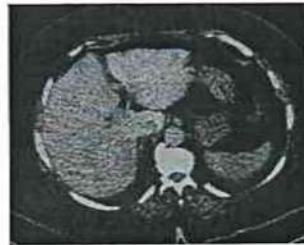
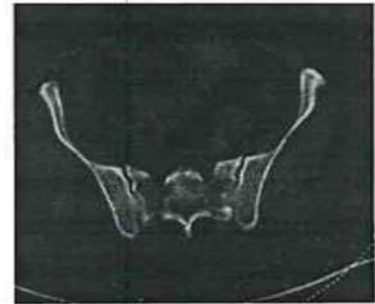


Name	Mrs. Laila Hassan Hussein Hassan		
Ref. By Prof. Dr.	Ashraf Mounir	Date	29/6/2026
ID	9926011151	Acc. No.	240322184

- Normal appearance of the bony skeleton with no evidence of any abnormal avid focal tracer uptake seen.

Abdomen and Pelvis:

- Multiple bilobar FDG avid hypodense focal hepatic lesions, the largest seen measures 3.6cm with SUVmax 6.7
- Normal size, parenchymal tracer uptake and CT texture of both kidneys; spleen; pancreas & both adrenals with no focal lesions or dense tracer fixing foci seen.
- Heterogenous low grade FDG avid omento peritoneal thickening with fat smudging, scattered nodules SUV max~2.7.
- Deep umbilicus low grade FDG avid nodule measures about 1.1 cm.
- No pathologically enlarged or dense tracer fixing LNs seen.
- No ascites, abdominal or pelvic collections noted.
- Normal appearance of pelvic organs.



Opinion:

- The current study shows:
 - Clear right MRM bed.
 - Bilobar FDG avid HFLs.
 - Low grade FDG avid pleural based right pulmonary consolidative patch
 - Heterogenous low grade FDG avid omento peritoneal thickening with fat smudging, scattered omental nodules as well as deep umbilicus nodule.
 - For MRI correlation.
 - Mixed FDG avid lytic and sclerotic osseous deposits.
 - Follow up PET/CT as clinically warranted would be advisable.

Thank you for referral,

Dr. Omnia Talaat, MD.
Professor of Nuclear medicine
NCI - Cairo University

02/07/26 11:09:46 AM

مركز فيصل : 111 شارع الملك فيصل - الهرم - الجيزة
مركز حدائق الهرم : ٢٠٠ م يمين البوابة الثالثة (منفرج)
فرع المنصورة (ميت عمر) : ٧٣ شارع الحرية - أمام مصنع الفول والنسوج
فرع المنيا : المليحة الجديدة - مول سيتي سكيب - الدور الأرضي



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19173



المركز الرئيسي : ٢٥١ شارع يوسف - السيدة زينب - القاهرة
مركز مصر الجديدة : ٨٧ شارع الميري - الخليفة المأمون - روكسي
مركز المعظم : مجمع جادون ش ٩ - أمام مسجد الحسن الثاني المعظم - خلف بناية مهين
الفرع الإداري : أول فيصل ٤٨١ برج السلام أمام كوبري فيصل

PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name : الملكة نوال بنت فيصل	Request No : 13004896
Patient No : 2561302101606	Request Date : 15/02/2022
Path No : 420/2022	Verification Date : 07/03/2022
Sex : Female	Financial Category : Urgent - Special
Age : 65 Y	Department : Out Patient
Referring Doc :	IHC No :
	Registered by : marwa mohamed yousif

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data

US guided biopsy from right breast malignant looking lesion

Nature of the specimen

tru cut biopsy

Gross Examination

- Multiple grayish white tissue cores up to 2cm , totally submitted
- Sections prepared from paraffin block for immune staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone 4B5, & Ki67 clone (30-9)

Microscopic examination

- Histological type: Invasive duct carcinoma
- Grade (differentiation + nuclear pleomorphism + mitosis): GII (3+2+1)
- Angiolymphatic No
- Ductal carcinoma in situ: No
- Biomarkers:
 - ER: Strong nuclear reaction in 95% of tumor cells (Allred score 8/8)
 - PR: Moderate nuclear reaction in 40% of tumor cells (Allred score 5/8)
 - Her-2: No membrane reaction in tumor cells (score 0)
 - Ki-67: 15 % nuclear reaction
- Appropriate tissue control were reviewed

Diagnosis

- US guided biopsy from right breast malignant looking lesion, tru cut biopsy
- Invasive duct carcinoma, grade II
- Biomarkers

Verified by
 walaa ghoniem

Printing Date: 14/03/2022 09:29:11

Page 1 of 2

Printed By: marwa mohamed

Patient Name: **Wafaa Ghonaim**
 Patient No: **2561302101604**
 In No: **420/2022**
 Sex: **Female**
 Age: **65 Y**
 Referring Doc:

Request No: **13004896**
 Request Date: **15/02/2022**
 Verification Date: **07/03/2022**
 Financial Category: **Out Patient**
 Department: **Out Patient**

Registered by: **marwa mohamed yousse**

IMMUNOHISTOCHEMISTRY REPORT

2+ Positive (Allred score 8/8)
 2+ Positive (Allred score 5/8)
 2+ Negative (score 0)
 67-15 % nuclear reaction

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $\leq 10\%$ of tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense in $\leq 10\%$ of tumor cells.

Prof. Dr. Gamal Abdelatty
 Dr. Abdelrahman Ibrahim

Dr. Wafaa Ghonaim

Verified by
 wafaa ghonaim

Printing Date: 14/03/2022 09:29:11

Page 2 of 2

Printed By: **marwa mohamed yousse**

Name: *Lalla Hassan* Date: *15-03-2022*

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Reported case of right breast cancer.
- Left sided fissural based nodule measuring 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD seen insitu.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- Left ischial and right femoral sclerotic foci mostly bone islands.

OPINION:

- Reported case of right breast carcinoma.
- Left fissural based pulmonary nodule likely non-specific.
- No definite hepatic or osseous metastatic deposits.

Dr. Mohammed Mohsen, MSc.

Dr. M. Goma, M.D.